



Situation Report

Crisis in Eastern Democratic Republic of the Congo

1 - 30 November 2025

Highlights

On 15 November 2025, the Government of the Democratic Republic of the Congo (DRC) and the March 23 Movement (M23) and Congo River Alliance (AFC), signed a peace roadmap in Doha, Qatar, mediated by international actors. Despite these diplomatic efforts, violence by non-state armed groups persisted throughout November, continuing to drive displacement across North Kivu, South Kivu, and Ituri.

Hostilities targeting civilian infrastructure—including a maternity ward and a displacement camp—disrupted essential health services. The destruction and damage to critical infrastructure continues to impede the delivery of specialized sexual and reproductive health (SRH) and gender-based violence (GBV) programmes.

Furthermore, humanitarian access and operations were severely limited due to consistent, high-level security incidents, with 37 incidents reported in eastern DRC during November.¹

In November 2025, more than 108,000 individuals were received with critical SRH services and over 165,000 people were reached with GBV prevention and response activities by UNFPA and its partners.



5,500,000

Total people affected²



1,200,000

Women of reproductive age³



269,550

Estimated pregnant women³



933,730

People targeted w/ SRH services



459,000

People targeted w/ GBV programmes

¹ [OCHA, Democratic Republic of the Congo - Eastern region: Overview of incidents against humanitarian workers \(Jan - Nov 2025\)](#).

² OCHA Protection Cluster. February 2025.

³ Estimated figures are based on the Minimum Initial Services Package (MISP) for Sexual and Reproductive Health in Humanitarian Settings calculator.

Situation Overview

The humanitarian situation in eastern DRC continues to be characterized by a complex landscape of persistent violence, shifting access constraints, and severe funding shortages. The crisis is driven primarily by intense conflict involving various armed actors. Conflict related sexual violence remains widespread, as armed groups continue to use rape as a weapon of war.

In November, the signing of the Doha Framework Agreement between the DRC government and M23, aimed to create a ceasefire framework, begin negotiations for political settlement, and reduce hostilities in eastern DRC. A broader US-brokered peace deal between the DRC and Rwanda addressing M23 backing was delayed amid continued fighting and mutual accusations of ceasefire violations.

Displacement, mainly in eastern DRC, remains among the largest worldwide. As of 30 November, approximately 3.7 million people are internally displaced in North Kivu, South Kivu, Ituri and Tanganyika.⁴

The situation is further exacerbated by recurring epidemics, which continue to pose serious public health risks. The DRC is currently experiencing its worst cholera outbreak in the past 25 years, with 64,427 reported cases since the start of the year, including 1,888 deaths.⁵ Cholera is particularly dangerous during pregnancy, as severe dehydration can rapidly threaten the lives of both the mother and fetus. Infection significantly increases the risk of miscarriage, stillbirth, preterm birth, and low birth weight, and in severe cases may lead to maternal death. In eastern DRC, the risk of cholera is especially acute, as widespread displacement has created poor water and sanitation conditions while access to health services remains severely limited.

UNFPA Response

Sexual and reproductive health:

In November 2025, more than 108,000 individuals were reached with critical SRH services. This effort included distributing life-saving supplies, such as interagency reproductive health kits (IARH) and safe delivery equipment to over 20 UNFPA-supported health facilities, as well as conducting community awareness and information sessions about reproductive health and rights.

In collaboration with the National Programme for Reproductive Health (PNSR) and implementing partner, Heal Africa, UNFPA initiated training for 45 provincial clinical mentors through the UK Foreign Commonwealth and Development Office (FCDO) funded project. The training was conducted in Masisi, Goma, and Beni territories, and aims to support critical services, including basic and comprehensive emergency obstetric and newborn care, to reduce preventable maternal deaths.

In addition, 20 medical practitioners, including anesthetic nurses, obstetricians, gynecologists, and general practitioners from Masisi and Goma territories, commenced a three-month hands-on session with UNFPA implementing partner, Heal Africa, to strengthen their capacity in obstetric surgeries, including cesarean sections and fistula repairs.

⁴ [OCHA. DR Congo: Humanitarian key figures as of 30 November 2025.](#)

⁵ [UNICEF. Democratic Republic of the Congo cholera outbreak is declared country's worst in 25 years. 8 December 2025.](#)

With the upsurge in the numbers of maternal deaths and the observed inaccuracies in primary audits conducted, as part of the accelerated maternal death plan under the UNFPA-FCDO funded project, the National Programme for Reproductive Health (PNSR) conducted a secondary review of maternal deaths to move beyond the limited, often inaccurate, information from primary reviews to determine the true, underlying cause of death, identify preventability, and trigger system-wide improvements. All maternal deaths recorded at least 60 days before November 2025 in the health zones of Goma, Karisimbi, Nyiragongo, and Mweso were included in this secondary review.

Gender-based violence:

In November 2025, more than 165,000 individuals were reached with GBV prevention, mitigation, and response activities. UNFPA and its partners provided integrated services across nine One-Stop Centers and eight safe spaces for women and girls, encompassing psychosocial and individual psychological support, occupational therapy, and vocational training. Interventions also prioritized community awareness regarding GBV risks and mitigation, alongside guidance on non-violent communication and emotional management to foster healthier relationships. These activities were designed to bolster the resilience and self-empowerment of women and girls, facilitating their successful reintegration into their communities. Prevention efforts included community mobilization focused on GBV education and the referral of cases for survivors. Furthermore, capacity-building initiatives trained 30 organizations in GBV case management and 50 women-led organizations in providing legal and socio-economic support to ensure the delivery of holistic, high-quality care.

Adolescents and youth:

Services delivered through youth-friendly spaces reached 1,075 adolescents and young people (598 females and 477 males). A total of 527 young people received specialized psychosocial support, and 46 young women and adolescent girls were provided with dignity and hygiene kits. Furthermore, 24 participants from 12 partner youth-led or youth-rights organizations participated in capacity-building activities covering SRH, leadership, youth, peace and security, protection, and livelihoods





108,425

People reached with SRH services
61% female, 39% male



65

Health facilities supported



165,097

People reached with GBV prevention,
mitigation and response activities
51% female, 49% male



8

Safe spaces for women
and girls supported



200

Non-food items (such as dignity
kits) distributed to individuals



229

Youth spaces supported



98

Reproductive health kits
provided to service delivery
points to meet the needs of
190,000 people



50

People reached with
humanitarian cash and
voucher assistance for
GBV and SRH

Coordination Mechanisms

Gender-based violence:

In November 2025, the GBV Area of Responsibility (AoR), co-led by UNFPA and Feminine Solidarity for Peace and Development (SOFEPADI), together with 41 partners—predominately national organizations—implemented a holistic response for GBV survivors across 35 health zones. Service coverage was concentrated in priority areas of Ituri, while North Kivu and South Kivu experienced critical under-coverage as a result of ongoing conflict and access limitations. Of the 407,233 individuals targeted, 168,395 received services, leaving nearly 60% of the target population unreached. Of those that received services, 72% were female and 28% were males.

Awareness-raising activities account for most interventions. The provision of specialized services, such as case management, psychosocial support, legal aid, and socio-economic activities remains limited, restricting survivors' access to comprehensive care. To address these gaps, the GBV AoR has recommended to prioritize access strategies and deploy mobile response teams in North and South Kivu, strengthen referrals and specialized services, scale up interventions addressing the specific needs of children and adolescent girls, and to continue investment in data quality to inform evidence-based programming.

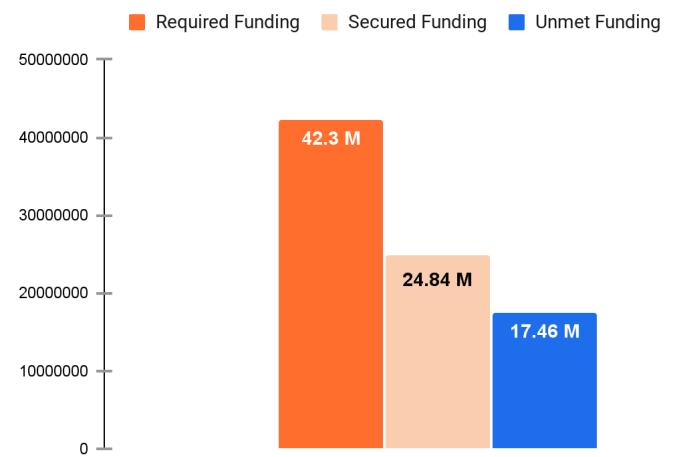
Sexual and reproductive health:

In November, UNFPA coordinated partners of the SRH Working Group by issuing technical guidance to improve service delivery and support frontline responders operating under extreme constraints. UNFPA, in consultation with the members of the SRH Working Group also contributed to the development of the 2026 Humanitarian Needs and Response Plan for DRC, providing analysis on SRH, and supporting the broader assessment conducted by the Health Cluster.

Members of the SRH Working Group, in collaboration with the provincial branches of the PNSR and the National Programme for Adolescent Health (PNSA), revitalized the Provincial Permanent Multisectoral Technical Committee for Family Planning (CTMP) in North Kivu. The CTMP is a technical body responsible for coordinating and promoting access to contraceptive services and reproductive health initiatives in the DRC, with the aim of strengthening family planning and reducing maternal mortality. Following its reactivation, the CTMP convened meetings to define strategic axes and actions with its four key committees—advocacy; adolescent and youth; religion; and logistics—aimed at strengthening family planning services in the North Kivu.

Funding Status

UNFPA is appealing for **US \$42.3 million in 2025** to scale-up and deliver life-saving SRH and GBV services to 1.8 million people across the eastern provinces of the DRC. During November, UNFPA received approximately **US \$1.81 million** in new contributions, bringing the total amount to **US \$24.84 million secured in 2025**. However, this amount reflects only **58.7% of the financial requirements of the appeal**, leaving a **funding gap of approximately US \$17.46 million**. Additional and timely resources are urgently required to sustain and expand critical SRH and GBV interventions for women, girls, and other vulnerable populations in North Kivu, South Kivu, and Ituri. Without further support, UNFPA's ability to maintain essential, life-saving services will be constrained, placing the health, rights, and dignity of women and girls at risk.



Disclaimer: Funding available is based on cash received during the current year as well as funding rolled over from previous years, and transfers from/to other UNFPA departments. It does not include funds from agreements that have been signed but not yet received.

Current Donors

- European Civil Protection and Humanitarian Aid Operations (ECHO)
- Government of Japan
- Government of Italy
- Government of Canada
- Government of Norway
- UK Foreign, Commonwealth & Development Office (FCDO)
- UN Central Emergency Response Fund (CERF)
- UNFPA Emergency Fund / Humanitarian Thematic Fund
- Democratic Republic of Congo Humanitarian Fund (DRC HF)
- Democratic Republic of Congo / National Fund for Reparation of Victims of Sexual Violence Related to Conflict

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